

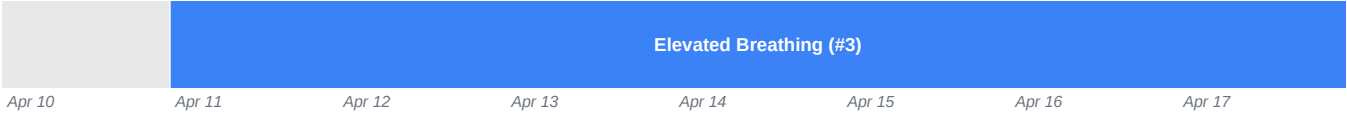
CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: EG Period: April 10 to April 17, 2024 Report Date: April 18, 2024 Coverage: Bed: 143/192h (74.5%)
Chair: 91/192h (47.4%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 8 days from Apr 10 to Apr 17 ■ HR ■ Breathing



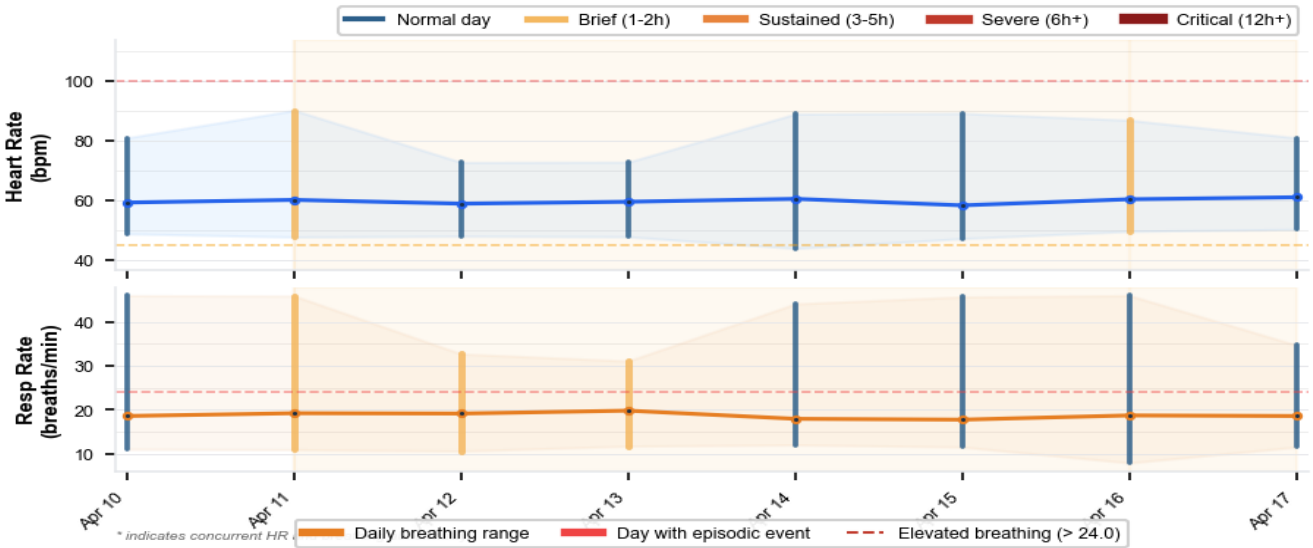
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
12	14h	Very High Breathing	2	100%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Very High Breathing	46 brpm	1h	1h	1	19 brpm	Apr 16
2	High Breathing	46 brpm	2h	1h	2	19 brpm	Apr 11
3	Elevated Breathing	44 brpm	11h	2h	1	19 brpm	Apr 11 to Apr 17

Clinical Pattern Observations:

- Daytime pattern:** Episodes concentrated during waking hours.
- Worsening trajectory:** 2 → 12 events vs prior period.



Red bars indicate days with detected episodic events.

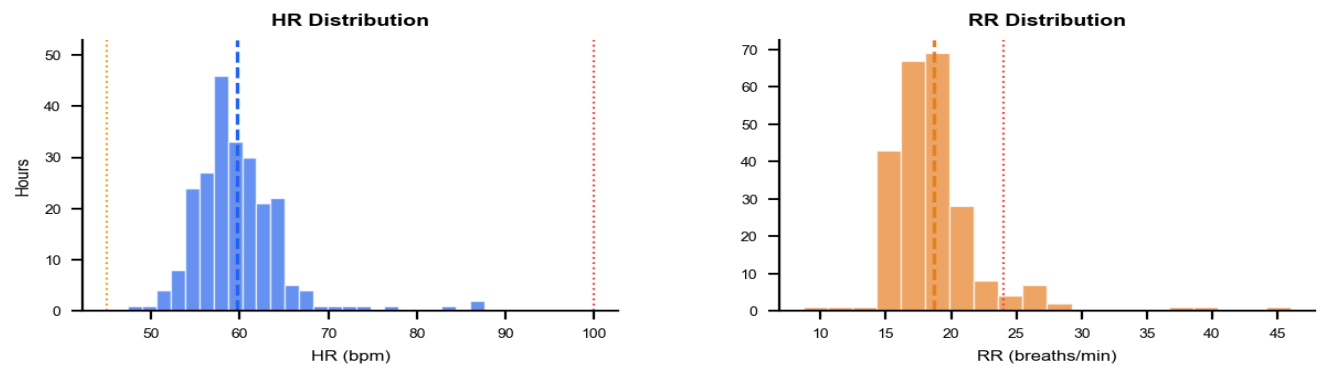
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

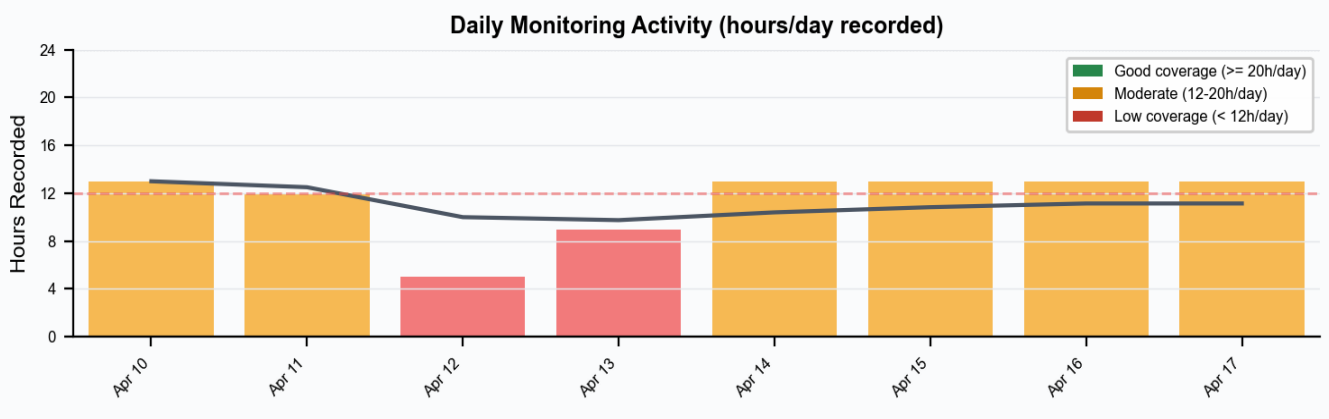
- Very Low HR < 40 bpm
Very High HR > 110 bpm
- Low HR < 45 bpm
Elevated Breathing > 24 brpm
- Elevated HR > 95 bpm
High Breathing > 30 brpm
- High HR > 100 bpm
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.